



Team Focused CPR (Optional)

Criteria for Death / No Resuscitation
Review DNR / MOST Form

YES

NO

AT ANY TIME

**Return of
Spontaneous
Circulation**



**Go to
Post Resuscitation
Protocol AC 10**

Begin Continuous CPR Compressions <i>Push Hard (≥ 2 inches)</i> <i>Push Fast (100 - 120 / min)</i> <i>Change Compressors every 2 minutes</i> <i>(sooner if fatigued)</i> <i>(Limit changes / pulse checks ≤ 10 seconds)</i>
Ventilate 1 breath every 6 seconds 30:2 Compression:Ventilation if no Advanced Airway Monitor EtCO2 if available
First Arriving BLS / ALS Responder Initiate Compressions Only CPR Initiate Defibrillation Automated Procedure if available Call for additional resources
Second Arriving BLS / ALS Responder Assume Compressions or Initiate Defibrillation Automated / Manual Procedure Place BIAD DO NOT Interrupt Compressions Ventilate at 6 to 8 breaths per minute

Decomposition Rigor mortis Dependent lividity Blunt force trauma Injury incompatible with life Extended downtime with asystole
Do not begin resuscitation
Follow Deceased Persons Policy

BLS

Third Arriving Responder
BLS or ALS

ALS

Establish Team Leader (Hierarchy) Fire Department or Squad Officer EMT First Arriving Responder
Rotate with Compressor To prevent Fatigue and effect high quality compressions Take direction from Team Leader
Fourth / Subsequent Arriving Responders Take direction from Team Leader
Continue Cardiac Arrest Protocol AC 3

Establish Team Leader (Hierarchy) EMS ALS Personnel Fire Department or Squad Officer EMT First Arriving Responder
A Initiate Defibrillation Automated Procedure Establish IV / IO Protocol UP 6 Administer Appropriate Medications Establish Airway with BIAD if not in place
P Initiate Defibrillation Manual Procedure Continuous Cardiac Monitoring Establish IV / IO Protocol UP 6 Administer Appropriate Medications Establish Airway with BIAD if not in place
Continue Cardiac Arrest Protocol AC 3

Team Leader

ALS Personnel
Responsible for patient care
Responsible for briefing / counseling family

Incident Commander

Fire Department / First Responder Officer
Team Leader until ALS arrival
Manages Scene / Bystanders
Ensures high-quality compressions
Ensures frequent compressor change
Responsible for briefing family prior to ALS arrival



Team Focused CPR (Optional)

Pearls

- This protocol is optional and given only as an example. Agencies may and are encouraged to develop their own.
- Team Focused Approach / Pit-Crew Approach recommended; assigning responders to predetermined tasks. Refer to optional protocol or development of local agency protocol.
- Efforts should be directed at high quality and continuous compressions with limited interruptions and early defibrillation when indicated.
- **DO NOT HYPERVENTILATE:** If no advanced airway (BIAD, ETT), compression to ventilation ratio is 30:2. If advanced airway in place, ventilate 10 breaths per minute with continuous, uninterrupted compressions.
- **Do not interrupt compressions to place endotracheal tube. Consider BIAD first to limit interruptions.**
- **Passive oxygenation optional in agencies practicing Team Focused Approach / Pit-Crew Approach.**
- Reassess and document BIAD and / or endotracheal tube placement and EtCO₂ frequently, after every move, and at transfer of care.
- **IV / IO access and drug delivery are secondary to high-quality chest compressions and early defibrillation.**
- **IV access is preferred route. Follow IV or IO Access Protocol UP 6.**
- **Defibrillation:** Follow manufacture's recommendations concerning defibrillation / cardioversion energy when specified.
 - Charge defibrillator during chest compressions, near the end of 2-minute cycle, to decrease peri-shock pause.
 - Following defibrillation, provider should immediately restart chest compressions with no pulse check until end of next cycle.
- Success is based on proper planning and execution. Procedures require space and patient access. Make room to work.
- Discussion with Medical Control can be a valuable tool in developing a differential diagnosis and identifying possible treatment options.